

Prior Authorization Request - Inpatient Admission

Health Plan

Humana Medicare Advantage. Plan ID: H5216-014-000. Request type: standard pre-service review.
Date of request: 2026-10-04

Member Information

Member: Gerald Fitzpatrick DOB: 1959-03-12 (age 67) Member ID: MBR-30417
Address: 812 Orchard Ridge Rd, Akron, OH 44313
Phone: (330) 555-0142 Sex: M
Primary insurance: Humana Medicare Advantage (primary). Secondary: none on file.

Requesting Provider

Dr. Anita Rao, MD -- Cardiac Electrophysiology
NPI: 1578043926
Practice: Heartland Cardiac Electrophysiology Associates
Phone: (330) 555-0170 Fax: (330) 555-0171

Requested Service

Requested service: Elective inpatient admission, implantation of a transvenous implantable cardioverter defibrillator (ICD)
CPT 33249 ICD-10: I42.0 (Dilated cardiomyopathy); I50.22 (Chronic systolic HF)
Planned admit date: 2026-11-10
Level of care requested: INPATIENT
Place of service: Hospital, inpatient (acute care).

Submission Contact

Submitted through the provider portal by Pamela Duarte, Authorization Coordinator.
Callback: (330) 555-0170. Supporting clinical documentation attached (8 pages including this request).

Cardiology Consultation - Heart Failure Clinic

Date of visit: 2026-09-16. Patient: Gerald Fitzpatrick Age/Sex: 67 M

Referred by: Dr. L. Mbeki (PCP) for evaluation of primary-prevention ICD candidacy.

Reason for visit

Follow-up of dilated cardiomyopathy with chronic heart failure with reduced ejection fraction (HFrEF).

History of present illness

67-year-old man with a non-ischemic dilated cardiomyopathy, first recognized after an admission for decompensated heart failure earlier this year. He reports dyspnea after walking about one block on level ground or climbing one flight of stairs, two-pillow orthopnea, and ankle swelling by the end of the day. He is comfortable at rest. No syncope or presyncope. No palpitations, no ICD shocks (no device in place). Weight is stable on diuretic. He has not needed an ED visit or admission for HF since the spring. Symptom burden has been stable over the last 3 months: marked limitation of ordinary activity, comfortable at rest, consistent with NYHA functional class III.

Cardiac history

Ischemic evaluation: left heart catheterization 2026-07-21 without obstructive coronary disease (report on file). No myocardial infarction, CABG or PCI in the past 12 months, and none before that either. No stent. No history of sustained VT, VF or cardiac arrest.

Past medical history

Heart failure as above. Hypertension, long-standing. Chronic kidney disease stage 3a (baseline eGFR in the high 40s). Remote appendectomy. Cataract extraction OU 2021. Seasonal allergies. Former smoker, quit 2008, 10 pack-years. Alcohol: 1-2 beers on weekends; counseled. No illicit drug use.

Exam

BP 118/72, HR 70 regular, SpO2 97% RA, BMI 29. JVP about 8 cm. Lungs clear. S3 present, soft apical MR murmur. Trace bilateral ankle edema. Warm extremities.

Assessment and plan

Dilated cardiomyopathy, non-ischemic, symptomatic HFrEF, NYHA III, on guideline-directed therapy (see medication list). LVEF remains severely reduced on the latest echocardiogram (page 3). Candidate for primary-prevention ICD. Referred to EP for implantation. Continue current regimen, daily weights, 2 g sodium diet. Return in 6 weeks.

Transthoracic Echocardiogram Report

Study date: 2026-08-12. Indication: cardiomyopathy, reassessment of LV function on therapy.

Patient: Gerald Fitzpatrick DOB: 1959-03-12 Sonographer: K. Yamada, RDCS. Interpreting physician: R. Castellano, MD, FASE.

Findings

Left ventricle: moderately to severely dilated (LVIDd 6.3 cm, LVIDs 5.6 cm). Wall thickness normal. Global hypokinesis with no discrete regional wall motion abnormality. No LV thrombus on contrast-enhanced views.

Left ventricular ejection fraction 28% by TTE (biplane method of discs, apical 4- and 2-chamber).

Diastolic function: restrictive filling pattern, E/e' 17.

Left atrium: moderately enlarged, LA volume index 44 mL/m².

Right ventricle: normal size, mildly reduced longitudinal function, TAPSE 15 mm.

Valves: mild-to-moderate functional mitral regurgitation. Mild tricuspid regurgitation. Aortic valve trileaflet, no stenosis.

Estimated RVSP 38 mmHg. IVC 2.0 cm with less than 50% collapse. No pericardial effusion.

Impression

1. Dilated left ventricle with severely reduced systolic function, LVEF 28%.
2. Mild-to-moderate functional MR. 3. Elevated filling pressures. 4. No thrombus.

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Cardiac Catheterization Report

Procedure date: 2026-07-21. Operator: T. Brannigan, MD. Access: right radial.

Indication: new cardiomyopathy with reduced EF; define coronary anatomy before device decisions.

Hemodynamics

RA mean 9 mmHg. PA 42/22, mean 30 mmHg. PCWP 21 mmHg. Aortic pressure 124/70. LVEDP 22 mmHg. Fick cardiac index 2.1 L/min/m².

Coronary angiography

Left main: normal. LAD: smooth, mild luminal irregularity, maximum 20% proximal stenosis. LCx: normal. RCA: dominant, normal. No obstructive coronary artery disease in any vessel. TIMI 3 flow throughout.

Intervention

None performed. No stent placed. Radial hemostasis band removed without incident.

Impression

Angiographically normal-to-minimal coronary arteries. Findings support a non-ischemic etiology for the patient's dilated cardiomyopathy. Elevated biventricular filling pressures. Continue medical therapy.

Medication Reconciliation - Heart Failure Clinic

Reconciled 2026-09-16 with patient and pharmacy fill history.

Allergies: sulfa (rash).

Guideline-directed medical therapy for HFrEF

Drug	Dose	Start date	Status
sacubitril/valsartan	97/103 mg BID	2026-02-02	active (titrated up 04-2026)
carvedilol	25 mg BID	2026-02-02	active (titrated up 05-2026)
spironolactone	25 mg daily	2026-03-02	active
dapagliflozin	10 mg daily	2026-03-16	active
furosemide	40 mg daily	2026-01-20	active, dose adjusted PRN

Other medications

amlodipine	5 mg daily	2019	active
atorvastatin	20 mg nightly	2022	active
omeprazole	20 mg daily	2023	active

Comment

Patient has remained on optimal tolerated doses of ARNI, beta blocker, MRA and SGLT2 inhibitor continuously since early February 2026, a total of 8 months of guideline-directed medical therapy at today's date. Adherence good per pharmacy refills; no missed fills. No drug intolerance. Potassium and creatinine monitored at each titration.

ECG Report and Laboratory Results

12-lead ECG, 2026-09-16

Sinus rhythm, rate 71. PR 206 ms (first-degree AV block). QRS 104 ms, no bundle branch block. QTc 452 ms. Nonspecific T-wave flattening laterally. No atrial fibrillation or flutter. No pathologic Q waves. Read by: S. Okonjo, MD.

Laboratory results, collected 2026-09-16

Test	Result	Reference
Sodium	139	135-145 mmol/L
Potassium	4.5	3.5-5.0 mmol/L
BUN	29 (H)	8-23 mg/dL
Creatinine	1.5 (H)	0.6-1.2 mg/dL
eGFR	48 (L)	>60 mL/min/1.73m2
NT-proBNP	2,140 (H)	<450 pg/mL
Hemoglobin	13.1	13.5-17.5 g/dL
TSH	1.9	0.4-4.5 mIU/L
HbA1c	5.6%	

Comment: CKD stage 3a, stable over three measurements since April 2026. TSH normal. Iron studies adequate.

Shared Decision Making Visit Note - ICD

Date: 2026-09-24. Duration: 38 minutes. Clinician: Dr. M. Aldaine, cardiology (not the implanting physician).

Patient: Gerald Fitzpatrick. Accompanied by: spouse.

Purpose

Formal shared decision making encounter before any decision on primary-prevention ICD implantation.

Decision tool

Reviewed the patient decision aid 'Should I get an ICD? A guide for people with heart failure' (evidence-based, patient-facing decision aid, booklet and short video). Patient and spouse viewed the video in clinic and walked through the booklet with the clinician.

Discussion

Discussed the absolute reduction in risk of sudden cardiac death with an ICD versus medical therapy alone, the chance of appropriate and inappropriate shocks, device infection, lead problems, bleeding, pneumothorax, battery replacement, and the option of declining the device. Discussed driving and activity guidance. Teach-back performed; patient restated risks and benefits correctly.

Outcome

Gerald values the chance of avoiding sudden death and prefers to proceed with ICD implantation. Questions answered. Decision made jointly. This visit took place before the planned implant.

Electrophysiology Surgeon Operative Plan

Date: 2026-09-30. Surgeon: Dr. Anita Rao, MD, cardiac electrophysiology.

Patient: Gerald Fitzpatrick.

Planned procedure

Insertion of a transvenous single-chamber ICD (CPT 33249), left subclavian approach under conscious sedation, with defibrillation testing per protocol. Pre-op: hold nothing; INR not applicable (no anticoagulant).

Indication

Primary prevention of sudden cardiac death in non-ischemic dilated cardiomyopathy with chronic systolic heart failure.

Expected hospital course

Anticipated inpatient stay: 2 midnights, for post-implant rhythm monitoring on telemetry, pocket and lead-position surveillance with a repeat chest film, device interrogation, and renal function and volume status monitoring given CKD and diuretic dosing. Discharge on post-op day 2 if device checks are normal.

Comorbidities relevant to peri-procedural risk

Chronic heart failure, hypertension, and CKD stage 3 (eGFR 48).

Pre-procedure assessment

Patient is clinically stable: euvolemic, NYHA III unchanged over 3 months, no admission since spring. No condition limiting life expectancy to under one year, no severe neurologic impairment, no atrial fibrillation (sinus rhythm on ECG), no active infection. No conditions that would rule out an ICD were identified.